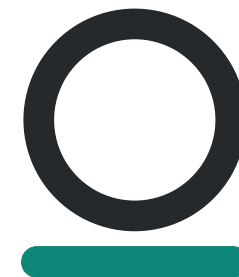


TIMEPIE LONGEVITY FORUM · BIOHACKER TRACK · SHANGHAI 2026



BIOHACK.IT

HACKING BIOLOGY

An open laboratory for longevity.

Turning thousands of individual self-experiments into structured, comparable, open data — with safety engineered in, not bolted on.

Open source · AGPL-3.0 · Non-profit — Hacking Biology

THE GAP

Longevity intervention is running far ahead of longevity measurement.

People already adopt off-label drugs, peptides, HBOT, plasmapheresis. But the protocol lives in spreadsheets, forum posts and chat prompts — not comparable, not followable, not readable by a physician.

130+

biomarkers a serious protocol tracks —
blood, DNA methylation, DEXA

$N = 1$

an isolated self-experiment carries almost
no statistical power

0

shared representation layer for protocol &
biomarker across people and labs

Bryan Johnson has a lab and a team. Everyone else has a spreadsheet. A thousand N-of-1 on the same protocol, made comparable, are a different scientific object — and it doesn't exist yet.

WHO IS EXPOSED

The beginner who copies is the person most at risk.

PRIMARY USER

The experienced biohacker

Designs, self-administers, iterates — but the results today are just anecdotes: claimed, not measured, impossible to compare.

HIGHEST RISK

The beginner who copies

Reads a forum thread or an influencer hyping a compound and starts taking it — no baseline, no safety markers, no measurement.

RECIPIENT, NOT USER

The treating physician

Today receives, at best, a handwritten sheet. Deserves a legible, complete view of what is taken and why.

The person who copies an off-label compound off a forum thread — without knowing to monitor kidney and liver — is exactly the case where the platform protects health.

THE MECHANISM

Follow and copy the people who get results — and see their biomarkers.

The social dynamic that powers copy-trading in finance, applied to health. The value is not the single datapoint; it is the ability to follow, compare and copy, anchored to structured, public biomarkers rather than anecdote.

- ◆ **A protocol becomes something you can subscribe to** — and it carries its safety markers with it.
- ◆ **Every compound and intervention gets a thread** anchored to real, comparable data.
- ◆ **A living community is the only quality control** that actually works on self-reported data.

THE PLATFORM

Document your protocol. Measure it. Share it.

01 — DOCUMENT

What you take & do

Substances, therapies, exercise, nutrition — with dose, timing and real cycle schemas. Create from scratch, or copy any public protocol with one click.

02 — MEASURE

Efficacy & safety

Biomarkers with reference, optimal and safety ranges, plus biological clocks (PhenoAge, epigenetic, glycation). Upload past lab reports, get your history in graphs.

03 — SHARE

Open by default

Publish your protocol and its outcomes. Follow and copy the people who get results, seeing their public biomarkers — the community is the quality control.

Open source, free, non-profit — built on the community that already exists, not a forum rebuilt from scratch.

DESIGN PRINCIPLES

Rigor is a design decision, not a disclaimer.

- ◆ **Describes, never prescribes.** Suggests what to measure, never what to take or how much.
- ◆ **Every claim graded to its evidence.** Human clinical → observational → animal → in vitro → anecdotal.
- ◆ **The LLM writes words, never numbers.** Extraction, mapping and thresholds are deterministic and reproducible.
- ◆ **It fails loudly.** Never drops an item silently — it shows the diff between input and output.
- ◆ **It stops instead of guessing.** When data is insufficient, it says so — no approximate index.
- ◆ **Sensitive data, heightened consent.** The most identifying data is shared only behind an explicit, informed gate.

DATA MODEL

Three questions, deliberately kept apart.

TestingProtocol

What I measure, and when. A versioned set of measures and timepoints.

TreatmentPlan

What I take and do. Versioned interventions — self-managed or physician-assigned.

Goal

What I want to change. Mapped onto the Hallmarks of Aging.

Substance → SafetyRule → Biomarker → Measurement → Dashboard

“What you take” and “how you are” are the same data structure, read from two sides.

ARCHITECTURE

One platform, one model.

24 capabilities, built in the open and grown by community review. Six families:

Protocols

create · copy · import · versioned

Blood Layer

biomarkers · clocks · deterministic

Safety guardrails

baseline · inheritance · dose check

Community & Studies

follow/copy · cohorts · N-of-1

Open Data & Standards

LOINC/UCUM · FHIR/OMOP · clonable

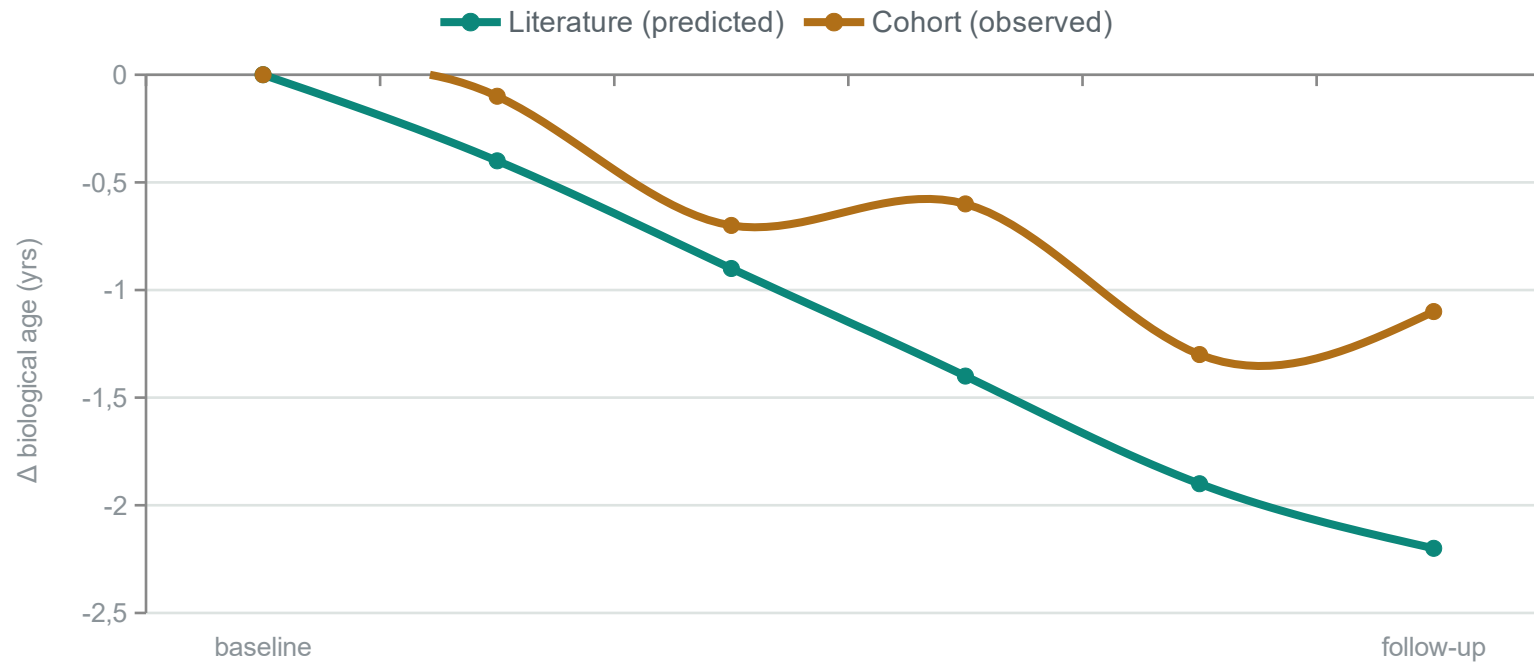
AI, Evidence & Provenance

AI4L · Evipedia · attribution

The Blood Layer and the Daily Log ship first — useful to a single person, with no community required.

WHAT NO ONE ELSE CAN BUILD

Literature predicts -2.2 years. The 340 who did it show this.



THE ORIGINALITY

**Outcome ↔
Biomarker**

Anyone can draw the first line — it is in the literature. Only a platform with a living community and shared, standards-coded biomarkers can draw the second, and put them side by side.

Illustrative. The link between an outcome and its biomarker is what makes the two columns comparable.

HARM REDUCTION

Follow a protocol — inherit its safety.

- ◆ **Mandatory baseline** before you can follow a protocol at all — plus an explicit risk acknowledgment.
- ◆ **Safety markers inherited automatically** from every active compound — safety is never opt-in.
- ◆ **Dose sanity check** catches the extra zero — plausibility validation of a number, not clinical advice.
- ◆ **Overdue is a safety gap** “rapamycin active for 94 days, liver panel overdue by 34.”

This is the module that justifies the project. Biohackers already take the risk today — without any of it.

PHASE 1 · THE PRODUCT YOU CAN DESCRIBE IN PUBLIC

Upload your past lab reports. Get your clinical history in graphs. Free.

A HARD BOUNDARY

The LLM writes words, never numbers

Extraction, mapping and thresholds are deterministic and reproducible. The model only produces the plain-language narration — generated once and cached.

BUILT FOR AGGREGATION

LOINC + UCUM + method, every value

The only architecture compatible with releasing open data. Three ranges per analyte: laboratory reference, longevity-optimal, safety.

A sentence you can say on television without pronouncing the word “biohacking”, with no regulatory exposure — and it works with a single user and no community.

BUILT FOR HACKERS · OPEN BY DEFAULT

Share your protocol. Share your outcomes. In public.

Public is the point

Server-hosted, always on, public by default. Your protocol and its biomarker outcomes are meant to be seen, followed and copied — how the community learns faster than any of us alone.

Your data becomes evidence

Every value you share, standards-coded, joins an open dataset on interventions no trial will ever fund. Contributing is the invitation.

Agent Access · MCP

A read-only token so you — or anyone you allow — can query the data with an agent: Claude, Cursor, a Nostr bot. No competitor will ship this.

Walled gardens are impossible by construction: AGPL means anyone running it as a service must open the source. Fork it.

BUILD NOTHING THAT ALREADY EXISTS

Standards, not silos.

Speak the standards

HL7 FHIR internally; OMOP CDM export for research — so what you produce is real research data with proper coding, not another wellness silo.

Stand on open shoulders

Evidence from Evipedia (Forever Healthy, CC BY 4.0); AI reviews via the AI4L audit framework; component reuse from get-based — all attributed, notified, contributed back.

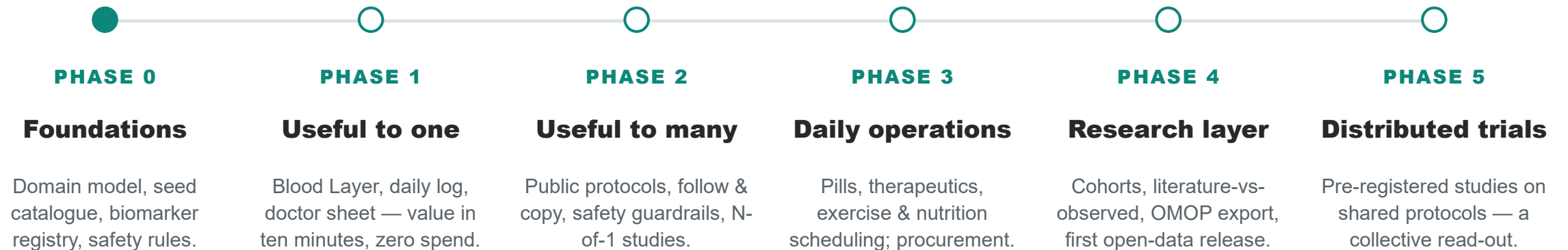
Existing community

The forum is not rebuilt: Rapamycin News is already there. Link first, cohorts second — never the reverse.

Open data released only with cohort thresholds and declared provenance. A small clean dataset beats a large uncomparable one.

FROM ONE PERSON TO A DISTRIBUTED TRIAL

An open, grassroots, distributed longevity trial — producing data.



THE INVITATION



Everyone buys anti-aging. **Almost no one measures it.**

Let's measure it — together, in the open.

Open infrastructure and open data for the longevity community — fork it, run it local, own your data.
The community already exists; now it gets its instrument.

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